



Obstetrical Labor and Childbirth

Assessment

History:

- Due date of LMP
- Time contractions started & how often
- Rupture membranes
- Time / amount of any vaginal bleeding
- Sensation of fetal activity
- Past medical and delivery history
- Gravida / Para status
- Medications
- If known high risk pregnancy

Signs & Symptoms:

- Episodic pain
- Vaginal discharge or bleeding
- Crowning or urge to push
- Meconium
- Urge to defecate

Differential:

- Abnormal presentation:
 - Buttock
 - Foot
 - Hand
- Prolapsed cord
- Placenta previa
- Abruptio placenta
- Premature labor

Clinical Management Options

P	P	P	P	P	P	• Delivery Checklist
L	L	L	L	L	L	• See Clinical Procedures for Birthing and Position Complications, and APGAR Scoring
1	2	3	4	5	6	• Oxygen, target SpO₂ to 92-96% • When the newborn's mouth appears over the perineum, immediately suction mouth then nose. • Allow newborn to nurse and massage the fundus to promote uterine contraction and prevent/control postpartum bleeding
						• Vascular access with Isotonic Crystalloid titrated to effect for vaginal hemorrhage • Tranexamic Acid for hypotension due to significant hemorrhage following delivery or delayed placenta delivery
						• Blood Product Transfusion and Calcium Chloride for severe postpartum hemorrhage

Consult Online Medical Control As Needed

Pearls:

- If stable, skin to skin contact between mother and baby is very important for healthy development. Delay clamping of umbilical cord for 3-5 minutes.
- Refer to drug formulary charts for all medication dosing for both adults and pediatric patients.
- **Contact OLMC with all indicated Complications of Labor**
 - Breech Delivery
 - Shoulder Dystocia
 - Prolapsed umbilical cord
 - Uterine inversion
 - Postpartum hemorrhage
- Document all times (delivery, contraction frequency, and length). Record APGAR at 1 minute and 5 minutes after birth.
- Post-partum hemorrhage defined as blood loss > 1000mL or > 500mL with signs/symptoms of hypotension. The perineum should be checked for bleeding from vaginal tears. Bleeding should be controlled by direct pressure over the laceration.
- The most common cause of post-partum hemorrhage is uterine atony due to prolonged labor, or multiple gestations